HCP Perception Survey: Opdivra in non-small cell lung cancer (EU, 2023)

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Executive Summary

Between June and September 2023, 300 oncologists in Germany, France, Italy, Spain, United Kingdom completed a perception survey on Opdivra (nivoltrimab), a PD-1 inhibitor, in non-small cell lung cancer.

Overall HCP sentiment index for Opdivra in Germany: 0.27 (scale 0 to 1, assessed 2023). Respondents viewed the brand negatively.

Methodology

Respondents were oncologists who treat at least 17 patients per month with non-small cell lung cancer. The sample size was n=300. Interviews lasted 23 minutes and were conducted online.

Results are weighted to national prescribing volumes. Statistical significance was assessed at the 95% confidence level.

Dose modification guidance for Opdivra is generally followed, but oncologists said that practical monitoring of pneumonitis depends on local capacity and patient travel distance.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Peer-to-peer education and congress data were cited as the most trusted sources when oncologists evaluate new evidence on PD-1 inhibitor agents.

The unmet need in non-small cell lung cancer persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

Cost-effectiveness submissions typically model first-line use, and payers question the durability of benefit beyond the trial follow-up period.

Supply and logistics are not a concern for Opdivra, though cold-chain handling and nurse-led administration add workload in busy clinics.

Guideline committees are expected to review the evidence base for non-small cell lung cancer again next year, which could change eligibility criteria and referral patterns.

Combination therapy and sequencing strategies remain debated; respondents noted limited head-to-head evidence between Opdivra and competing agents.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

Treatment burden, including monitoring visits and missed maintenance doses, was described as a key factor in long-term adherence and persistence.

Clinical guidelines in Germany recommend first-line use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Respondents expect real-world persistence on Opdivra to be lower than in trials, mainly because of missed maintenance doses and the complexity of managing infusion reactions.

Key Findings

54% of respondents considered the efficacy of Opdivra clinically meaningful versus the current standard of care, with the strongest support in refractory disease use.

Oncologists in Germany rated Opdivra tolerability profile as negatively, citing pneumonitis and immune-related adverse events as the main concerns among patients with non-small cell lung cancer.

Respondents highlighted the mechanism of action of Opdivra (blocks the pd-1 receptor to restore t-cell activity) as a differentiator in non-small cell lung cancer, although the dosing schedule complicates adherence.

Prescribing intent for Opdivra among oncologists was high, with 33% planning to increase use in the next 12 months.

Adherence concerns centred on treatment burden over long durations, particularly in patients with comorbidities and polypharmacy.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Dose modification guidance for Opdivra is generally followed, but oncologists said that practical monitoring of immune-related adverse events depends on local capacity and patient travel distance.

Treatment burden, including monitoring visits and treatment burden over long durations, was described as a key factor in long-term adherence and persistence.

Biosimilar and generic competition in adjacent classes is shaping price expectations, even where Opdivra has no direct substitute.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Respondents expect real-world persistence on Opdivra to be lower than in trials, mainly because of treatment burden over long durations and the complexity of managing rash.

Real-world evidence from registries suggests that outcomes in routine practice are broadly consistent with phase 3 trial results, although persistence and discontinuation rates are higher outside clinical trials.

Oncologists emphasised that patient-reported outcomes and quality of life are increasingly important when choosing between PD-1 inhibitor options.

Cost-effectiveness submissions typically model refractory disease use, and payers question the durability of benefit beyond the trial follow-up period.

Subgroup analyses suggest that benefit with Opdivra is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

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Tolerability and Safety Perceptions

Tolerability was assessed against pneumonitis and immune-related adverse events. Oncologists reported managing these events with dose adjustment and supportive care.

Safety perception differed by experience: respondents with more than 17 treated patients rated Opdivra more favourably.

Peer-to-peer education and congress data were cited as the most trusted sources when oncologists evaluate new evidence on PD-1 inhibitor agents.

Regional variation within Italy is notable: large centres report faster uptake and broader use than smaller regional hospitals.

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Treatment burden, including monitoring visits and monitoring requirements, was described as a key factor in long-term adherence and persistence.

Oncologists emphasised that patient-reported outcomes and quality of life are increasingly important when choosing between PD-1 inhibitor options.

Supply and logistics are not a concern for Opdivra, though cold-chain handling and nurse-led administration add workload in busy clinics.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Registry data show that outcomes in non-small cell lung cancer depend heavily on early diagnosis, access to specialist care and adherence to maintenance therapy.

Safety monitoring focused on rash and fatigue, with dose interruption or discontinuation used for severe adverse events.

Real-world evidence from registries suggests that outcomes in routine practice are broadly consistent with phase 3 trial results, although persistence and discontinuation rates are higher outside clinical trials.

Respondents expect real-world persistence on Opdivra to be lower than in trials, mainly because of monitoring requirements and the complexity of managing fatigue.

Conclusions

Opdivra is perceived negatively in Germany. Priorities for the brand team are treatment burden over long durations and evidence generation for non-small cell lung cancer.

Treatment burden, including monitoring visits and missed maintenance doses, was described as a key factor in long-term adherence and persistence.

Supply and logistics are not a concern for Opdivra, though cold-chain handling and nurse-led administration add workload in busy clinics.

Combination therapy and sequencing strategies remain debated; respondents noted limited head-to-head evidence between Opdivra and competing agents.

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Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Medical education priorities include recognising rash early, communicating realistic expectations, and coordinating with primary care.

Clinical guidelines in Italy recommend refractory disease use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Segment	Respondents	Sentiment index
High-volume prescribers	120	0.31
Low-volume prescribers	180	0.24

Synthetic document generated for the Watson prototype. All brand names and figures are fictional.